Eye History — OSCE Checklist

Deakin MD2 · BB8 · Ophthalmology

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching. Use a person-centred approach (Calgary-Cambridge). Ask* **unilateral vs bilateral** *for every symptom.*

1. Opening

☐	Introduce self, confirm patient ID, consent
☐	Open question: "What's brought you in about your eyes today?"

2. Presenting complaint

	Ask about	Example	Significance
☐	Laterality	One eye or both?	
☐	Onset and duration	Sudden or gradual? How long?	Sudden = urgent
☐	Previous episodes	Has this happened before?	Recurrent uveitis, HSV
☐	Vision change	Reduced acuity, blurring, diplopia, field defect	Acute loss = red flag
☐	Flashes and floaters	New flashes, floaters, "curtain" over vision	Retinal tear / detachment
☐	Redness	Where? Diffuse or around the cornea?	Unilateral red eye = red flag
☐	Pain	Severity, constant vs intermittent	Constant pain = red flag
☐	Discharge	Watery, purulent, crusting	Viral vs bacterial conjunctivitis
☐	Photophobia	Light sensitivity	Keratitis, uveitis, meningitis
☐	Sensation	Gritty, foreign-body feeling, itch, dryness	FB, abrasion, allergy, dry eye

3. Eye-specific background

☐	Vision aids	Glasses, contact lenses (overnight wear?)	Contact lens keratitis
☐	Previous eye surgery	Laser, cataract, intravitreal injections, retinal detachment repair	Recent procedure = red flag
☐	Previous trauma	Any past eye injury	
☐	Last review	Last optometry / ophthalmology review	

4. Trauma history*

☐	High-speed injury	Drilling, hammering, grinding, explosion	Beware painful impact then painless eye — penetrating FB
☐	Sharp object	Glass, nail, metal	Globe rupture
☐	Contaminant	Plant material	Fungal keratitis
☐	Chemical	Acid or alkali	Alkali = worst; beware the "white eye"
☐	PPE	Eye protection worn?	

5. Systemic review

	Ask about	Think of
☐	Fever, nausea, vomiting, headache	Acute angle closure, orbital cellulitis
☐	Jaw claudication, scalp tenderness, age >50	Giant cell arteritis
☐	Weight loss, night sweats, rashes, recent viral illness	Systemic or infective cause
☐	Vesicular rash / girdle pain on face	HZO — check Hutchinson's sign (nose tip)
☐	Neurological: speech, vertigo, cognition, balance, weakness, sensory change, diplopia	Stroke, space-occupying lesion

6. Past history, medications, family and social

☐	PMH	Diabetes, CVD, hypertension, autoimmune / connective tissue disease, immunosuppression, HSV/VZV
☐	Medications	COCP, corticosteroids, biologics, α- and β-blockers, anticholinergics, eye drops
☐	Allergies	Drug allergies †
☐	Family history	Glaucoma (incl. angle closure), macular degeneration, strabismus / amblyopia
☐	Social	Smoking, occupation and PPE, **driving**
☐	Risk factors	Age, myopia / hypermetropia, prematurity, previous eye trauma

7. Close

☐	Ideas, concerns, expectations (e.g. impact on driving, work)
☐	Summarise, check accuracy, invite questions, thank patient